FAX TRANSMITTAL

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Specialty: Allergy / Immunology
Sample Name: Chronic Sinusitis

HISTORY:, I had the pleasure of meeting and evaluating the patient referred today for evaluation and treatment of chronic sinusitis. As you are well aware, she is a pleasant 50-year-old female who states she started having severe sinusitis about two to three months ago with facial discomfort, nasal congestion, eye pain, and postnasal drip symptoms. She states she really has sinus problems, but this infection has been rather severe and she notes she has not had much improvement with antibiotics. She had a CT of her paranasal sinuses identifying mild mucosal thickening of right paranasal sinuses with occlusion of the ostiomeatal complex on the right and turbinate hypertrophy was also noted when I reviewed the films and there is some minimal nasal septum deviation to the left. She currently is not taking any medication for her sinuses. She also has noted that she is having some problems with her balance and possible hearing loss or at least ear popping and fullness. Her audiogram today demonstrated mild high frequency sensorineural hearing loss, normal tympanometry, and normal speech discrimination. She has tried topical nasal corticosteroid therapy without much improvement. She tried Allegra without much improvement and she believes the Allegra may have caused problems with balance to worsen. She notes her dizziness to be much worse if she does quick positional changes such as head turning or sudden movements, no ear fullness, pressure, humming, buzzing or roaring noted in her ears. She denies any previous history of sinus surgery or nasal injury. She believes she has some degree of allergy symptoms.,PAST MEDICAL HISTORY: ,Seasonal allergies, possible food allergies, chronic sinusitis, hypertension and history of weight change. She is currently 180 pounds.,PAST SURGICAL HISTORY:, Lower extremity vein stripping, tonsillectomy and adenoidectomy.,FAMILY HISTORY: , Strong for heart disease and alcoholism.,CURRENT MEDICATIONS: , DynaCirc.,ALLERGIES: , Egg-based products cause hives.,SOCIAL HISTORY: ,The patient used to smoke cigarettes for about 20 years, one-half pack a day. She currently does not, which was encouraged to continue. She rarely drinks any alcohol-containing beverages.,PHYSICAL EXAMINATION: ,VITAL SIGNS: Age 50, blood pressure is 136/74, pulse 84, temperature is 98.4, weight is 180 pounds, and height is 5 feet 3 inches.,GENERAL: The patient is healthy appearing; alert and oriented to person, place and time; responds appropriately; in no acute distress.,HEAD: Normocephalic. No masses or lesions noted.,FACE: No facial tenderness or asymmetry noted.,EYES: Pupils are equal, round and reactive to light and accommodation bilaterally. Extraocular movements are intact bilaterally. No nystagmus.,EARS: During Hallpike examination, the patient did not become dizzy until she would be placed back into sitting in the upright position. No nystagmus was appreciated; however, the patient did subjectively report dizziness, which was repeated twice. No evidence of any orthostatic hypotension was noted during the exam. Tympanic membranes were noted to be intact. No signs of middle ear effusion or ear canal inflammation.,NOSE:

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The patient appears congested. Turbinate hypertrophy is noted. There are no signs of any acute sinusitis. Septum is midline, slightly deviated to the left.,THROAT: There is clear postnasal drip. Oral hygiene is good. No masses or lesions noted. Both vocal cords move well to midline.,NECK: The neck is supple with no adenopathy or masses palpated. The trachea is midline. The thyroid gland is of normal size with no nodules.,LUNGS: Clear to auscultation bilaterally. No wheeze noted.,HEART: Regular rate and rhythm. No murmur noted.,NEUROLOGIC: Facial nerve is intact bilaterally. The remaining cranial nerves are intact without focal deficit.,PROCEDURE: , Fiberoptic nasopharyngoscopy identifying turbinate hypertrophy and nasal septum deviation to the left, more significant posteriorly.,IMPRESSION: ,1. Probable increasing problems with allergic rhinitis and chronic sinusitis, both contributing to the patient's symptoms.,2. Subjective dizziness, etiology uncertain; however, consider positional vertigo versus vestibular neuronitis as possible ear causes of dizziness, cannot rule out systemic, central or medication or causes at this time.,3. Inferior turbinate hypertrophy.,4. Nasal septum deformity.,RECOMMENDATIONS:, An ENG was ordered to evaluate vestibular function. She was placed on Veramyst nasal spray two sprays each nostril daily and even twice daily if symptoms are worsening. A Medrol Dosepak was prescribed as directed. The patient was given instruction on use of nasal saline irrigation to be used twice daily and Clarinex 5 mg daily was recommended. After the patients' ENG examination, we will see the patient back for further evaluation and treatment recommendations. In light of the patient's atypical dizziness symptoms, I cannot rule out other pathology at this time, and I informed her if there are any acute changes or problems with regards to her balance or any other acute changes, which she attributes associated with her dizziness, she most likely should pursue an emergent visit to the emergency room.,Thank you for allowing me to participate with the care of your patient.

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EXTERNAL EXAMINATION: , The body is that of a 62-inch, 112-pound white female who appears the recorded age of 41 years. The body is clad in a pink and white gown. Three pillows and a blanket are also received with the body. The scalp is covered in thick brown hair with flecks of gray. The irides are brown. There is bilateral tache noire. The eyelids are yellow and dry. The eyes have a sunken appearance. The ears and nose are normally developed. The mouth has partial natural dentition. The left upper first bicuspid through the molars are absent. The left lower bicuspid is absent. The right upper second molar is capped. The left lower first molar appears decayed. The teeth are otherwise in good repair. The lips and buccal mucosa have no trauma. The neck is unremarkable except for a 2.5 cm tracheostomy scar just above the suprasternal notch. The posterior portion of the neck is unremarkable and free of scars. An obliquely oriented 6 cm surgical scar is on the anterior left chest with an underlying, implanted, medical device. The breasts are pendulous and otherwise unremarkable. There is white powder underneath the breasts. A round, 8 mm scar is on the upper central abdomen. A horizontally oriented 2.5 cm linear scar is on the central upper abdomen. A faint, approximately 1 cm scar is on the right mid lateral abdomen. There are a few striae on the hips and lower abdomen. The external genitalia are normally developed and white powder covers the perineum. The labia are dry. The urethral meatus is visible and 3.5 mm in diameter. No objects or substances are in the vagina other than a slight amount of yellow-white discharge. The anus is patent and unremarkable. Faint, pink-white, flat, 1-2 cm scars are just above the superior portion of the gluteal cleft. A 2.5 cm, somewhat square shaped, brown macule is on the left buttock. There are no open and active decubitus ulcers. The upper extremities have flexion contractures with striae on the medial portions of the upper arms. The muscles of the extremities are atrophic. The lower extremities are partially shaved. The left fifth toe is absent. The skin on the back is intact. The spine has accentuated thoracic kyphosis and lumbar lordosis. The skin demonstrates tenting,RADIOGRAPHS: , Postmortem radiographs show radiopaque shadows extending from the periosteum of the femurs, left tibia, and right ischial tuberosity. Diffuse, severe osteoporosis is present. The 11th thoracic vertebral body has an endplate fracture. Degenerative joint changes are noted in the acromioclavicular joints, hips, right knee, left foot, and pelvis. The left fifth toe is amputated along with the distal portion of the left fifth metatarsal. Radiographs of anterior neck structures and iliac wings are not remarkable. Calculi are seen in the urinary tract. Staples are in the gallbladder bed.,INTERNAL EXAMINATION: , The muscles of the chest and abdominal wall are normally developed. The subcutaneous tissues are dry. The panniculus is 2.5-3 cm. In the left chest wall is an implanted medical device with a wire extending through subcutaneous tissues of the left neck and into the left scalp. A flat, four-prong electrical device is in the

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subgaleal area of the left scalp. A wire then further extends into the cranial cavity. The peritoneal cavity is unremarkable and dry. There are no intraperitoneal adhesions except for an adhesion of the anterior portion of the stomach to the anterior abdominal wall in the area of the previously described round abdominal scar. The organs are in the usual anatomic relations. The pleural cavities are dry. The lungs are well aerated. The pericardial sac is remarkable for a 1 cm focal area of anterior pericardial adhesion to the anterior portion of the right ventricle. There is some lateral adhesion of the right ventricle to the right lateral portion of the pericardial sac. No other adhesions are noted. The pericardial sac is dry. The diaphragm is intact. The sternum is unremarkable. The ribs have no trauma and are normally developed with somewhat prominent costochondral junctions.,CARDIOVASCULAR SYSTEM: , The pericardial sac is remarkable as previously described. The epicardial fat of the 255 gram heart is otherwise unremarkable. The root of the aorta has no atherosclerosis. The arch and descending aorta have minimal atherosclerosis (see attached CV pathology report).,RESPIRATORY SYSTEM: , The right and left lungs are 260 and 245 grams, respectively. The lungs have a normal number of lobes and have light pink-red outer surfaces. The bronchi are unobstructed. The well-aerated lung parenchyma is pink-red. There are no anthracosis, tumors, cysts, or infarcts. The upper lobe bronchi contain a scant amount pearlescent fluid. The proximal bronchi contain yellow pearlescent fluid. The lower lobe distal bronchi contain some scattered areas of yellow pearlescent fluid. The alveoli otherwise contain foamy, reddish-white fluid. The pulmonary arteries contain no emboli. The lower lobes have firm areas of partial consolidation with yellow-green pearlescent fluid. The firm area of the left lower lobe is ,4 x 4 x 3 cm. The right lung has scattered firm areas (<1cm).,HEMOLYMPHATIC SYSTEM: , The 215 gram spleen is covered in an intact, gray, somewhat wrinkled capsule. There are two hilar accessory spleens (1.4 and 1 cm in diameter). The splenic parenchyma is dark red-maroon and unremarkable. There is no interstitial fibrosis, tumors, cysts or infarcts. No enlarged lymph nodes are noted. The bone marrow of the lumbar vertebral bodies is red and soft.,GENITOURINARY SYSTEM:, The right and left kidneys are 100 and 130 grams, respectively. The right kidney has a central, 2-2.5 cm, obliquely oriented cleft/scar extending from the central renal pelvis to the upper lateral cortex. The brown-tan outer surfaces are otherwise slightly lobular and granular. The pelvis of the right kidney is mildly dilated. A 1 x 0.6 x 0.7 cm, green-brown stone is in the pelvis of the right kidney. The left renal pelvis has an approximately 0.5 x 0.6 x 1 cm, green-brown stone. The corticomedullary ratios are reduced. The pelvic fat is increased. The left ureter contains pearlescent fluid. The urinary bladder contains ,6 cc of brown-yellow fluid. A 3.8 x 1.2 x 1 cm, white-yellow, somewhat crescent shaped stone is within the lumen of the bladder. The uterus is present and

has a normal shape. The cervix is normally developed. The cervical os is large (coned) and contains mucoid fluid. There are a few minute nabothian cysts (<2 mm). A 2 cm, spherical leiomyoma is in the posterior portion of the uterine corpus. The endometrial cavity contains 3 to 4 mm thick, tan endometrium. The ovaries are present, firm and otherwise grossly unremarkable. The fallopian tubes are unremarkable except for a few adhesions of the fimbriated ends.,GASTROINTESTINAL SYSTEM:, The stomach contains 60 cc of green-brown fluid without any solid food fragments. The gastric mucosa is flat, congested, and green-gray. The gastric mucosa is congested. No ulcerations are noted. There are a few congested vessels with minute petechiae around the previously healed ostomy site. An 8 mm blood clot is on the gastric mucosa near the healed/healing gastrostomy site. The wall of the stomach is thin (<3mm). No perforations are noted. The esophagus is not remarkable with gray/pink mucosa. The bowel contains progressively formed feces with the rectum containing hard stool. The appendix is present, but is atrophic/small. The bowel has no perforations. An abundant amount of greenish liquid is in the duodenum. No foreign objects are noted.,HEPATOBILIARY SYSTEM:, The outer surface of the 965 gram liver is covered in a transparent intact capsule. There are very few inferior hepatic adhesions associated with an absent gallbladder. Surgical staples are imbedded in the area of the cystic duct. The hepatic parenchyma is brown-green with a slight pattern of congestion. The bile ducts and portal veins appear grossly unremarkable. No fibrosis, cysts or infarcts are noted. A yellow-white, round, 2 mm nodule is in the anterior portion of the right lobe of the liver.,ENDOCRINE SYSTEM: , The adrenals and pancreas are present and grossly unremarkable. The thyroid is mildly atrophic without nodules.,MUSCULOSKELETAL SYSTEM: , The upper and lower extremities are atrophic as previously described. The trunk musculature is atrophic. A 1 x 1.5 x ,1 cm area of induration/calcification extends from the anterior surface of the right femur. The anterior/lateral cortical surface of the distal right femur metaphysis is rough and irregular. The cortical bone of the lumbar and thoracic vertebral bodies is thin and soft. The iliac wings have no trauma or deformity.,NECK: , The strap muscles of the anterior neck have intact musculature with atrophy of the musculature on the right side. The right sternocleidomastoid is moderately atrophic. There are no hemorrhages. The larynx and piriform recesses contain yellow-tan, mucoid fluid. There is yellow-green, mucoid fluid on the base of the tongue and epiglottis. The larynx contains a scant amount of fluid. The thyroid and cricoid cartilages are intact. The hyoid bone is intact. The tongue is atrophic. There is a yellow-green dry crusted material on the surface of the tongue. The posterior pharyngeal musculature appears atrophic. There are no hemorrhages. A healed tracheostomy site is on the anterior trachea. The carotid arteries and jugular veins are not remarkable. The muscles and

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cervical vertebral bodies of the posterior neck are not remarkable. The spinal cord and column have no trauma. The posterior laminae are soft.,CENTRAL NERVOUS SYSTEM: , See neuropathology report,MICROSCOPIC EXAMINATION: , (Also see neuropathology and cardiovascular pathology reports),LUNGS, LOWER LOBES: Widespread bronchopneumonia. Intraalveolar debris and bacteria.,Intraalveolar foamy macrophages. Congestion. Edema.,UTERUS: Late proliferative endometrium leiomyoma.,ACCESSORY SPLEEN: Not remarkable.,COLON: Autolysis. Melanosis.,URINARY BLADDER: Chronic inflammation.,OVARIES: Corpora albicantia. Follicular cyst.,FALLOPIAN TUBES: Paratubal cyst. Congestion.,VAGINA: Vascular congestion.,ADRENAL GLANDS: Mild congestion.,STOMACH, GASTROSTOMY: Clotted blood.,LIVER: Focal nodular hyperplasia (single focus). Centrilobular congestion with steatosis,KIDNEYS: Tubular necrosis. Mild vascular congestion.,THYROID: Not remarkable.,PANCREAS: Early autolysis. Mild interstitial fibrosis.,EPIGLOTTIS: Focal ulceration with acute inflammation.,LARYNX, RIGHT ARYEPIGLOTTIC FOLD: Not remarkable.

Specialty: Bariatrics
Sample Name: Laparoscopic Gastric Bypass Consult - 1

HISTORY OF PRESENT ILLNESS: , I have seen ABC today. He is a very pleasant gentleman who is 42 years old, 344 pounds. He is 5'9". He has a BMI of 51. He has been overweight for ten years since the age of 33, at his highest he was 358 pounds, at his lowest 260. He is pursuing surgical attempts of weight loss to feel good, get healthy, and begin to exercise again. He wants to be able to exercise and play volleyball. Physically, he is sluggish. He gets tired quickly. He does not go out often. When he loses weight he always regains it and he gains back more than he lost. His biggest weight loss is 25 pounds and it was three months before he gained it back. He did six months of not drinking alcohol and not taking in many calories. He has been on multiple commercial weight loss programs including Slim Fast for one month one year ago and Atkin's Diet for one month two years ago.,PAST MEDICAL HISTORY: , He has difficulty climbing stairs, difficulty with airline seats, tying shoes, used to public seating, difficulty walking, high cholesterol, and high blood pressure. He has asthma and difficulty walking two blocks or going eight to ten steps. He has sleep apnea and snoring. He is a diabetic, on medication. He has joint pain, knee pain, back pain, foot and ankle pain, leg and foot swelling. He has hemorrhoids.,PAST SURGICAL HISTORY: , Includes orthopedic or knee surgery.,SOCIAL HISTORY: , He is currently single. He drinks alcohol ten to twelve drinks a week, but does not drink five days a week and then will binge drink. He smokes one and a half pack a day for 15 years, but he has recently stopped smoking for the past two weeks.,FAMILY HISTORY: , Obesity, heart disease, and diabetes. Family history is negative for hypertension and stroke.,CURRENT MEDICATIONS:, Include Diovan, Crestor, and Tricor.,MISCELLANEOUS/EATING HISTORY: ,He says a couple of friends of his have had heart attacks and have had died. He used to drink everyday, but stopped two years ago. He now only drinks on weekends. He is on his second week of Chantix, which is a medication to come off smoking completely. Eating, he eats bad food. He is single. He eats things like bacon, eggs, and cheese, cheeseburgers, fast food, eats four times a day, seven in the morning, at noon, 9 p.m., and 2 a.m. He currently weighs 344 pounds and 5'9". His ideal body weight is 160 pounds. He is 184 pounds overweight. If he lost 70% of his excess body weight that would be 129 pounds and that would get him down to 215.,REVIEW OF SYSTEMS: , Negative for head, neck, heart, lungs, GI, GU, orthopedic, or skin. He also is positive for gout. He denies chest pain, heart attack, coronary artery disease, congestive heart failure, arrhythmia, atrial fibrillation, pacemaker, pulmonary embolism, or CVA. He denies venous insufficiency or thrombophlebitis. Denies shortness of breath, COPD, or emphysema. Denies thyroid problems, hip pain, osteoarthritis, rheumatoid arthritis, GERD, hiatal hernia, peptic ulcer disease, gallstones, infected gallbladder, pancreatitis, fatty liver, hepatitis, rectal bleeding, polyps, incontinence of stool, urinary stress incontinence, or cancer. He denies cellulitis, pseudotumor cerebri, meningitis, or encephalitis.,PHYSICAL

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EXAMINATION: ,He is alert and oriented x 3. Cranial nerves II-XII are intact. Neck is soft and supple. Lungs: He has positive wheezing bilaterally. Heart is regular rhythm and rate. His abdomen is soft. Extremities: He has 1+ pitting edema.,IMPRESSION/PLAN:, I have explained to him the risks and potential complications of laparoscopic gastric bypass in detail and these include bleeding, infection, deep venous thrombosis, pulmonary embolism, leakage from the gastrojejuno-anastomosis, jejunojejuno-anastomosis, and possible bowel obstruction among other potential complications. He understands. He wants to proceed with workup and evaluation for laparoscopic Roux-en-Y gastric bypass. He will need to get a letter of approval from Dr. XYZ. He will need to see a nutritionist and mental health worker. He will need an upper endoscopy by either Dr. XYZ. He will need to go to Dr. XYZ as he previously had a sleep study. We will need another sleep study. He will need H. pylori testing, thyroid function tests, LFTs, glycosylated hemoglobin, and fasting blood sugar. After this is performed, we will submit him for insurance approval.

LABORATORY REPORT

Packet: 001

Analyte	Value	Units	Flag
Glucose	142	mg/dL	High
Hemoglobin	11.8	g/dL	Low
WBC	12.4	10*3/uL	High
Sodium	136	mmol/L	
Potassium	4.2	mmol/L	
Creatinine	1.4	mg/dL	High
BUN	24	mg/dL	High
Platelets	228	10*3/uL	
Hemoglobin A1c	7.6	%	High
TSH	2.1	mIU/L	